A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

14 May 2026

Miss Salena Ward
Melbourne Gastro Osophageal Surgery

Dear Miss Salena Ward,

Re: Ms Wendy Atkins
DOB: 16/01/1955 Phone: 0407 778 455
Address: 405/57 Stables Circuit, DONCASTER, VIC, 3108
Email: wende.atkins@gmail.com

Summary

- **Oesophageal and visceral hypersensitivity.**
- **Commence low-dose amitriptyline (10mg) after pending ECG review.**
- **Follow-up in eight weeks prior to overseas travel.**
- **No action required by you.**

I had the pleasure of conducting an in-person consultation with Wendy Atkins. As you are aware, she has a complex history of upper gastrointestinal symptoms spanning several years, which have become particularly distressing over the last four years following an attempt to cease long-term proton pump inhibitors (PPIs).

Symptoms and history of presenting complaint

Wendy's primary complaint is nocturnal burning pain located under the diaphragm, radiating through to her back and shoulder blade. These episodes occur twice nightly, forcing her to sleep propped up. The burning is invariably associated with significant bouts of belching lasting up to thirty minutes. She finds some relief with water, Gaviscon, and Zantac (nizatidine), as well as consuming carbohydrates.

She has a long history of PPI use but discontinued them due to significant side effects, including nausea, "brain fog", and word-finding difficulties. Attempts to restart Somac recently resulted in similar cognitive symptoms without providing significant relief for her burning pain. Wendy also describes several extra-oesophageal symptoms that she attributes to "seeping" reflux, including recurrent bronchitis, chronic sinus infections, and a frequent choking sensation or globus. She notes a change in her voice and occasional ear ringing.

Wendy has seen several specialists, including surgeons, regarding the possibility of a fundoplication. While a recent pH study apparently confirmed acid exposure, she does not have erosive oesophagitis. She has been appropriately cautioned her that surgery may not resolve her sensory symptoms and carries a high risk of gas-bloat syndrome.

Diet

Wendy follows a restricted diet. She identified salicylate sensitivity early on and avoids flavour-heavy, spicy, or vinegary foods. She drinks black tea with ice to avoid burning in her throat. She has largely replaced meat with roasted vegetables and salmon and avoids most fruits, with the exception of bananas.

All appointments and enquires
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Pager: 03 9387 1000
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Endoscopy locations
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Hobson Healthcare Sydenham
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St Vincent's Private Hospital
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Past medical history and medications

Wendy's medical history is significant for systemic lupus erythematosus (diagnosed 1996) and idiopathic cardiomyopathy, which has more recently been characterised as heart failure with reduced ejection fraction (HFrEF). She underwent a cholecystectomy in 2006 for similar symptoms of chest pain and vomiting, which provided only temporary relief. Other history includes a prior back injury and vitamin D deficiency. Her current medications include nizatidine (Zantac), Nevolet, Cubasol, pravastatin, ezetimibe, Jardiance, and vitamin D.

Social history, family history Wendy is a retired health and safety officer and former hairdresser. She lives with her husband and remains very active with her four children and seven grandchildren. Her father had bowel cancer and her mother suffered from similar refractory reflux symptoms until her death at age 97. She is a non-smoker and does not drink alcohol.

Examination On examination, she was comfortable. Abdominal palpation did not reveal any significant tenderness or masses, and she confirms she has no altered bowel habits or lower gastrointestinal symptoms.

Impression and plan

I agree with you I believe Wendy's symptoms are classic for oesophageal and visceral hypersensitivity. This explains why standard acid suppression with PPIs has been unsatisfying and why mechanical surgery is unlikely to provide the relief she seeks.

The goal of management is to "turn down" this neural signaling. I have proposed a trial of low-dose amitriptyline. I explained to Wendy that we are using this at a sub-clinical dose (10mg) for its neuromodulatory effects on the autonomic nervous system rather than for any antidepressant effect. I suspect the sedative side effect may also assist her in staying asleep through the night.

Given her history of HFrEF, I am cautious regarding the potential for amitriptyline to affect her cardiac rhythm, even at this minute dose. She is seeing her cardiologist, Dr Leighton Keeney, next week for a routine check-up including an ECG and Holter monitor. I have asked Wendy to provide me with the results of that ECG before she commences the medication.

Pending a satisfactory ECG, the plan is:

- **Amitriptyline:** Start at 5mg (half a 10mg tablet) at night for one week to ensure tolerability, then increase to 10mg.
- **Zantac:** Continue as required. As she has no erosive disease, there is no mandatory requirement for her to endure the side effects of a PPI.
- **Follow-up:** I will review her in eight weeks. This will allow enough time to assess the benefit of the medication before she departs for a six-week trip to Europe in mid-year.
- **Monitoring:** If her symptoms remain stable, I would suggest a surveillance gastroscopy in three to four years, but no urgent intervention is required.

Thank you again for referring this patient.

Yours sincerely,

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